

GI IMAGING (Radiology) — Revision Table

Bold = high-yield. Plain film + modalities + esophagus/stomach + intestine.

PLAIN FILM & MODALITIES	
Topic	Key points
Plain AXR	Supine AP standard; free air = ERECT CXR ; can't sit → lateral decubitus
SBO vs LBO	SBO: central, valvulae conniventes (stack of coins, cross full width), 3–6cm, numerous. LBO: peripheral, haustra (incomplete bands), >6cm, few
Pneumoperitoneum	Commonest = perforated PU ; curvilinear gas under R hemidiaphragm on erect CXR
Calcifications	Appendicolith (appendicitis), phlebolith, gallstone 20% (dense rim), pancreatic = chronic pancreatitis (L1) , dermoid = tooth, AAA = curvilinear, staghorn = renal
Contrast	Barium = best detail but CONTRAINDICATED if perforation ; Gastrografin = perforation/leak (aspirate → pulmonary oedema)
Modalities	EUS = tumour depth ; MRI = rectal cancer staging + MR enterography (Crohn's) ; CT = wall/staging/obstruction; PET = mets
Descriptive	Benign stricture = tapered ; malignant = shouldering (abrupt overhanging). Fat stranding = inflammation
ESOPHAGUS	
Condition	Imaging
Carcinoma	Irregular lumen + shouldered edges ; EUS/CT/PET staging
Peptic stricture	Lower end, short, smooth, tapered
Achalasia	Smooth tapered lower narrowing + dilated above, absent peristalsis (bird's beak) ; aspiration
Corrosive	Long stricture from aortic arch level
Scleroderma	Dilatation WITHOUT obstruction (lower 2/3)
Zenker / web	Zenker = posterior pharynx; web = cervical + Plummer-Vinson
Perforation	Pneumomediastinum; water-soluble contrast / CT oral contrast
STOMACH	
Condition	Imaging
Benign ulcer	Projects BEYOND contour, smooth deep crater, folds reach margin, Hampton's line
Malignant ulcer	Within contour, irregular shallow, folds DON'T reach, Carman meniscus sign
Gastric carcinoma	Irregular filling defect / focal thickening / linitis plastica (rigid stomach, lost folds)
GIST	Smooth round submucosal filling defect; 60–70% stomach
Lymphoma	Stomach = commonest GI site ; diffuse thickening + bulky nodes
Hiatus hernia	Sliding (GEJ above, reflux); rolling (fundus, GEJ competent); CXR retrocardiac air-fluid level
SMALL & LARGE INTESTINE	
Condition	Imaging
UC	Continuous from rectum, lost haustra (lead pipe), shallow ulcers, pseudopolyps, target sign, toxic megacolon ; stricture rare → cancer
Crohn's	Skip lesions, terminal ileum, deep ulcers → cobblestone, string sign, fistulae , fat stranding
Diverticular	Sigmoid commonest; diverticulitis = thick wall + fat stranding; stricture mimics cancer
Appendicitis	US/CT: >6mm non-compressible, thick wall, fat inflammation ; abscess = RIF mass
Volvulus	Sigmoid commonest; coffee-bean sign (AXR), whirl sign (CT)
Intussusception	Children = idiopathic (target, reduce by air/CO2 enema) ; adults = tumour lead point (CT sausage mass)
Mesenteric ischaemia	SMA occlusion, gas in wall, CTA thrombus. Ischaemic colitis = thumbprint sign (splenic flexure–sigmoid)
TB	Contracted caecum + dilated ileum; mimics Crohn's → biopsy
Carcinoma	Rectosigmoid (annular stricture, obstruct) + caecal (large, no obstruction, anaemia/weight loss) ; shouldered <6cm ; MRI rectal staging

Condition	Imaging
Lymphoma	Marked wall thickening WITHOUT narrowing (vs Crohn's)
Polyps	Malignant if >2cm, thick short stalk, irregular